

ADVERSE EVENT REPORT FORM - page 1 of 3 (synthetic)

Case reference: SYN-2025-0442

Patient initials: T.M. Age: 68 Sex: Male Weight: 79 kg Height: 175 cm

Ethnicity: not recorded

Relevant medical history:

- Type 2 diabetes mellitus (12 years)
- Diabetic nephropathy, eGFR 38 mL/min/1.73m²
- Hypertension
- Prior myocardial infarction (2019)

Relevant concomitant medication:

- Metformin 1 g twice daily
- Ramipril 10 mg once daily
- Atorvastatin 40 mg once daily
- Aspirin 75 mg once daily

ADVERSE EVENT REPORT FORM - page 2 of 3

Suspect product: Glcaris 10 mg tablets (dapagliflozin-class, fictional)

Indication: type 2 diabetes, add-on therapy

Dose and route: 10 mg once daily, oral

Therapy start date: 02 Jan 2025

Therapy stop date: 19 Jan 2025

Adverse event term: euglycaemic diabetic ketoacidosis

Event onset date: 17 Jan 2025

Description: The patient presented to the emergency department with a 2-day history of nausea, vomiting, abdominal pain and deep sighing breathing. Capillary glucose was 11.4 mmol/L, venous pH 7.19, bicarbonate 9 mmol/L, blood ketones 5.8 mmol/L. A diagnosis of euglycaemic DKA was made. Glcaris was stopped on admission. The patient was managed with an intravenous insulin and dextrose infusion and fluid resuscitation on the high-dependency unit.

ADVERSE EVENT REPORT FORM - page 3 of 3

Seriousness criteria: hospitalisation (5 nights); medically important

Life-threatening: No Death: No Congenital anomaly: Not applicable

Action taken with suspect drug: permanently withdrawn

Outcome: recovered without sequelae by 24 Jan 2025

Dechallenge: positive (ketoacidosis resolved after withdrawal)

Rechallenge: not performed

Reporter assessment of causality: probable

Reporter: Dr. Helen Asante, consultant endocrinologist

Reporter country: Ghana

Report submitted: 27 Jan 2025

Narrative note: patient had been fasting for a religious observance in the days before presentation, which may have contributed.